# RTT within 18 weeks

Outturn for September 2026 · forecast issued with data through August 2026

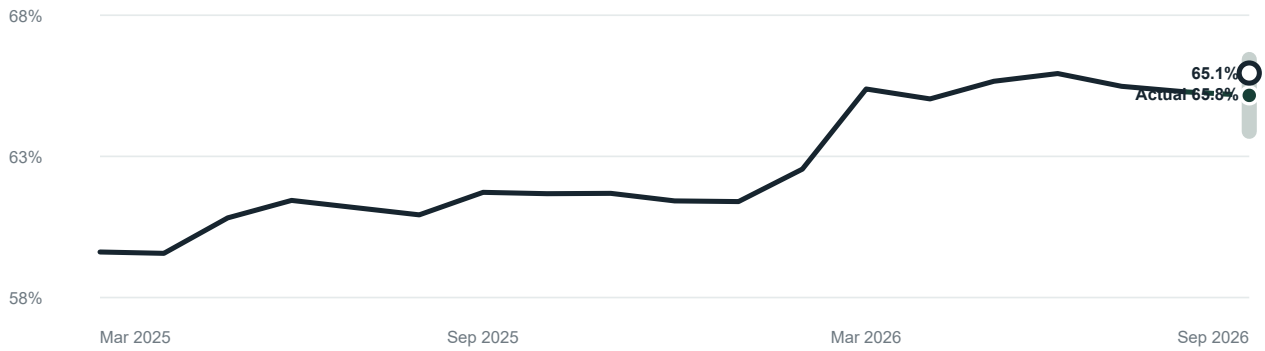
**The published result was 65.8%. It improved 0.6pp from the previous result and was 0.8pp better than outlook · inside the 80% range.**

| AT FORECAST · AUGUST 2026 | FORECAST · SEPTEMBER 2026 | ACTUAL |
|---|---|---|
| **65.2%** | **65.1%** | **65.8%** |
| Known when the forecast was made | 80% expected range 63.9%–66.3% | Improved 0.6pp from the previous result · 0.8pp better than outlook · inside the 80% range |
| | | Off course · 70% by March 2027 |

**Mar 2027 outlook at issue** Projected **66.6%** Target **≥70%** Model-implied chance **6%**

## Recent performance and current release

The vertical band is the 80% expected range.




## Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.




**Benchmark:** 92% constitutional standard. 3.8pp below milestone

**Data:** NHS England referral to treatment waiting times. **Targets:** Medium Term Planning Framework national target · Medium Term Planning Framework final target

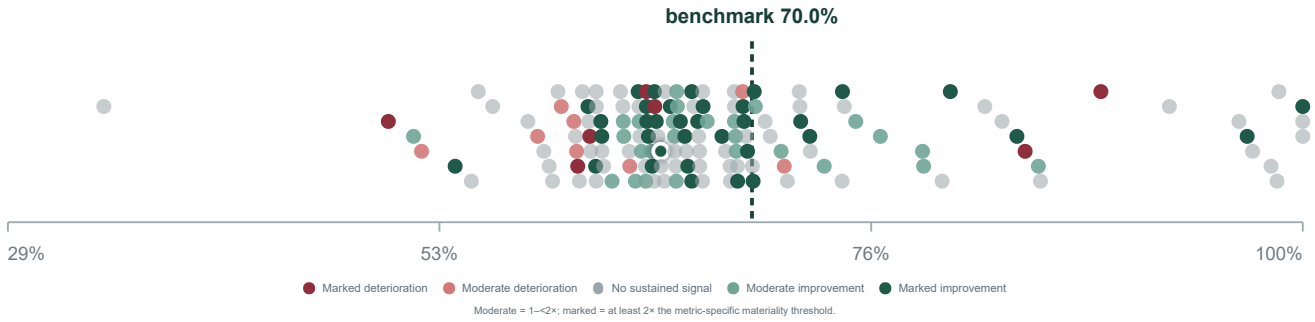
**Method:** Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 230 consecutive months. Accuracy is tested on 36 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 24 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

# RTT within 18 weeks

Distribution and trajectory watch · September 2026

## Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



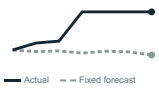

## Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the latest actual minus the fixed forecast for that same month; the six-month window checks persistence.

| 137 | 61 | 15 | 61 |
|---|---|---|---|
| providers assessed | sustained favourable | sustained adverse | without an active signal |

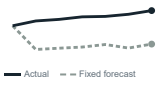
### Largest sustained favourable moves

**OXLEAS NHS FOUNDATION TRUST**
Latest 100.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



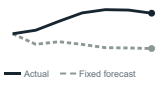

49.9pp
LATEST MONTH: ABOVE FORECAST

**COUNTESS OF CHESTER HOSPITAL NHS FOUNDATION TRUST**
Latest 69.8% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window




12.2pp
LATEST MONTH: ABOVE FORECAST

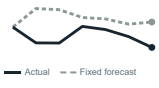
**MID CHESHIRE HOSPITALS NHS FOUNDATION TRUST**
Latest 69.5% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window




12.0pp
LATEST MONTH: ABOVE FORECAST

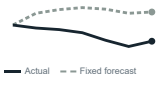
### Largest sustained adverse moves

**BRADFORD DISTRICT CARE NHS FOUNDATION TRUST**
Latest 50.2% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



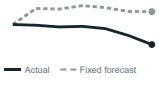

11.6pp
LATEST MONTH: BELOW FORECAST

**UNIVERSITY HOSPITALS OF NORTH MIDLANDS NHS TRUST**
Latest 61.2% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window




6.4pp
LATEST MONTH: BELOW FORECAST

**MIDLANDS PARTNERSHIP UNIVERSITY NHS FOUNDATION TRUST**
Latest 89.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window




6.3pp
LATEST MONTH: BELOW FORECAST

**Trajectory-watch rule.** Six actual months are compared with the corresponding path fixed before the six-month window. The displayed gap is the latest actual minus the fixed forecast for that same month. A signal requires a material latest-month gap and at least five of six months in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.